



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سهير احمد عثمان عبدالمحسن

المحافظة : القاهرة

الحي / المدينة : مصر القديمة

العنوان : منيل الروضة مضر
القديمة

تاريخ تسجيل الإستمارة : 22-09-2026

رقم البطاقة : 25706150102825 رقم التليفون : 01008837559

العمر(تاريخ الميلاد) : 15-06-1957 المهنة : حاصل علي بكالوريوس

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان الرئة

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان الرئة

خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6347] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة :
الوظيفة :
الوظيفة :

التوقيع :
التوقيع :
التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سهر احمد عثمان عبدالمحسن National ID: 25706150102825	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of lung cancer for Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name : الأستاذ / سهير احمد عثمان عبد المحسن	Request No. : 13006237
Patient No. : 25706150102825	Request Date : 02/05/2026
Path No. : 1271/26	Verification Date : 09/05/2026
Sex : Female	Financial Category : جوماتاف روسي - اقتصادي
Age : 68 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: PET-CT showing; Newly developed hypermetabolic bilateral pulmonary nodules as well as hilar and mediastinal nodal lesions ? metastatic, in a patient with history of right breast cancer underwent right mastectomy in 11-2024 as well as received radiotherapy and on hormonal treatment.

Nature of Specimen: Left lung mass, Tru-cut biopsy, collected on 02/05/2026, at 11:00 am, Cold ischemia time: 20 seconds.

Gross examination:

One grey white tissue core measured 1.3 cm. Totally submitted.

Microscopic examination:

Histopathological examination reveals core of lung tissue showing infiltration by tumor formed of malignant epithelial cells with enlarged hyperchromatic nuclei and moderate pleomorphism.

There is focal necrosis.

Diagnosis:

Left lung mass, Tru-cut biopsy:

- Metastatic carcinomatous deposits, likely of breast origin.
- Case for further (GATA-3, ER, PR, Her2 & SISH).

Signature

Dr. Walaa Ebrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

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Patient Name	: الأستاذ / سهير احمد عثمان عبد المحسن	Request No.	: 13006237
Patient No.	: 25706150102825	Request Date	: 02/05/2026
Path No.	: 1271/26	Verification Date	: 09/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - اقتصادي
Age	: 68 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: PET-CT showing; Newly developed hypermetabolic bilateral pulmonary nodules as well as hilar and mediastinal nodal lesions ? metastatic, in a patient with history of right breast cancer underwent right mastectomy in 11-2024 as well as received radiotherapy and on hormonal treatment. Nature of Specimen: Left lung mass, Tru-cut biopsy, collected on 02/05/2026, at 11:00 am, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; GATA-3 clone (L50-823), ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & SISH.

Results:

* IPT (All controls showed appropriate reactivity):

- GATA-3: Diffusely strong positive staining in tumor cells.
 - ER: Strong nuclear reaction in 100% of tumor cells (score 8/8).
 - PR: Weak nuclear reaction in 30% of tumor cells (score 4/8).
 - Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Not amplified.
- (Average Her2 copy No.: (1.58), Her2/CEP17 ratio: (1.23)).

Diagnosis:

Left lung mass, Tru-cut biopsy:

- Findings are consistent with metastatic carcinomatous deposits of breast origin.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.

Verified by :

Dr.Duaa Mohamed

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Patient Name الأستاذ / مهيير احمد عثمان عبد المحسن
Patient No. 25706150102825
Path No. 1271/26
Sex Female
Age 68 Y
Referreing Doc.

Request No. 13006237
Request Date 02/05/2026
Verification Date 09/05/2026
Financial Category جوستاف روسي - اقتصادي
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

(Average Her2 copy No.: (1.85), Her2/CEP17 ratio: (1.23)).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+(Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

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Patient Name	الأستاذة / سهير احمد عثمان عبد المحسن	Request No.	13006237
Patient No.	25706150102825	Request Date	02/05/2026
Path No.	1271/26	Verification Date	09/05/2026
Sex	Female	Financial Category	جوستاف روسي - اقتصادي
Age	68 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ebrahim Ghoneim.

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Dr.Duaa Mohamed

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جمهورية مصر العربية
بطاقة تحقيق الشخصية

سهير
احمد عثمان عبدالمحسن

٢٧ ش احمد مختار حجازي - منيل الروضة
مصر القديمة - القاهرة

١٩٥٧/٠١/١٥ ٢٥٧.٦١٥.١.٢٨٢٥
1V6627547

DMS
← علامه دوميستا
← رايبر انسا حقا لودن ← الواجهات DMS

٢٠٢١/٠٤ ٢٥٧.٦١٥.١.٢٨٢٥

حاصله على بكالوريوس علوم زراعية

مسلمة أنثى

البطاقة سارية حتى ٢٠٢٨/٠٢/٢١

١

06/01/2026

MDT FORM

Name:	سهر أحمد عثمان عبدالعظيم	MRN:	10584
Age:	69 Y	ID:	25706150102825
Menopausal Sta	Post	Phone:	01003005962
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	systemic therapy

Clinical Data:

BX:	IDC
ER:	8/8
PR:	4/8
HER2:	2+ (sligh)
KI67:	

R

L

Sonomammography:

Mets Workup:

History:

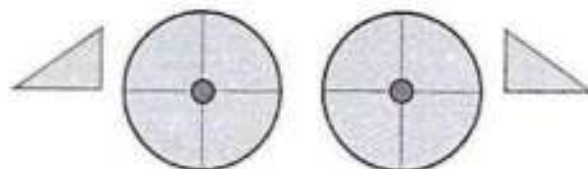
Breast, NOS: Infiltrating duct carcinoma, NOS MBC
HTN
Rt breast cancer since 2022 + single lung mets
Arimidex 2 years --> PD Faslodex in 5/2024 (4 months) in baheya but PD of breast mass
New biopsy: ER 7/8 PR 4/8 HER2: 1+
Rt MRM: ypT2 N1
Started aromazin 1/2025
PET CT 4/2026: new multiple lung and hilar nodules
Complaining from hoarsness of voice, cough
New biopsy ER 8/8 PR 4/8 Her2: 2+ not amplified
For MDT for Ibrance + Faslodex + Zometa / 6m

Recommendations:

Ibrance + Faslodex + Zometa / 6m

MDT Outcome & Plan

Ibrance + Faslodex + Zometa / 6m



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نوره مهندي					



Patient Name	: الأستاذ / سهيل احمد عثمان عبد المحسن	Request No.	: 13003078
Patient No.	: 25706150102825	Request Date	: 02/06/2024
Path No.	: SR-709/24	Verification Date	: 10/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 66 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Metastatic breast cancer.

Nature of Specimen: Block revision.

Gross Examination:

- One paraffin block labeled by (3782/22) was received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Negative.
- * Perineural invasion: Negative.
- * Angioinvasion: Negative.

Diagnosis:

Metastatic breast cancer, Block revision:

Invasive duct carcinoma, Grade II.

For IPT (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

DR.Menna Nabil

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Patient No.	: 25706150102825	Request Date	: 02/06/2024
Path No.	: SR-709/24	Verification Date	: 25/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 66 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Metastatic breast cancer.

Nature of Specimen: Block revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8 /8).
- PR: Moderate nuclear reaction in 25% of tumor cells (score 5/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 19% nuclear reaction of tumor cells.

Conclusion:

Metastatic breast cancer, Block revision:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 5 /8).
- Her2: Negative (score 0).
- Ki67: Positive in 19% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

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Patient No.	25706150102825	Request Date	02/06/2024
Path No.	SR-709/24	Verification Date	25/06/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	66 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Heba Hassan.

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DR.Menna Nabil

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L A B S

Collection Date: 29/04/2026

Review Date: 29/04/2026

Patient Name: الأستاذ/ سهر احمد عثمان

Patient No.: 3-40747

Accession No.: 326044154

Age: 68Y

HEMATOLOGY UNIT

Complete Blood Picture (CBC)

Test Name	Result	Unit	Reference Interval
Haemoglobin	13.70	g/dL	12 - 15
Hematocrit	39.0	%	36 - 46
Red cell count	4.80	$\times 10^6 / \mu\text{L}$	3.8 - 4.8
MCV	81.1	fL	83 - 101
MCH	28.5	pg	27 - 32
MCHC	35.1	g/dL	31.5 - 34.5
RDW	14.0	%	11.6 - 14
Platelet Count	197	$\times 10^3 / \mu\text{L}$	150 - 410
T.L.C	2.5	$\times 10^3 / \mu\text{L}$	4 - 10

Differential Count

Percent Values

Absolute Values

Basophils	0	%	0 - 2	0.0	$\times 10^3 / \mu\text{L}$	0.02 - 0.1
Eosinophils	2	%	1 - 6	0.1	$\times 10^3 / \mu\text{L}$	0.02 - 0.5
Stab	0	%	0 - 7	0.0	$\times 10^3 / \mu\text{L}$	
Segmented	45	%	40 - 80	1.1	$\times 10^3 / \mu\text{L}$	2 - 7
Lymphocytes	40	%	20 - 40	1.0	$\times 10^3 / \mu\text{L}$	1 - 3
Monocytes	13	%	2 - 10	0.3	$\times 10^3 / \mu\text{L}$	0.2 - 1
MPV	9.1	fL	8 - 12			

Comments : Normal Hb, RBCs show mild microcytosis.
Platelets are adequate.
Mild leucopenia.
Mild absolute neutropenia.



Reviewed By:

Dr. Sara Mohamed

PL Branch: النيل - الروقة

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Printed by: 326044154 29/04/2026

ا.د. أحمد كمال الدري
 ا.د. محمد أبو الهدي درويش
 ا.د. عمر حسين عمر
 ا.د. وحيد حسين طنطاوي
 ا.د. ياسر عبد العظيم عباس
 ا.د. أحمد محمد منيب
 أساتذة الأشعة التشخيصية
 كلية الطب - جامعة عين شمس

Patient Name: Sohair Ahmed Othman	Patient ID: 1336351
Dear Sir	Study Date: 22/04/2026
Acc No: 2690712	

F18- FDG PET/CT EXAMINATION

History:-

A 68-year-old female patient, presented with history of right breast cancer, underwent right mastectomy in 11-2024 as well as received radiotherapy and on hormonal treatment. Referred for follow-up PET/CT examination.

Procedure:-

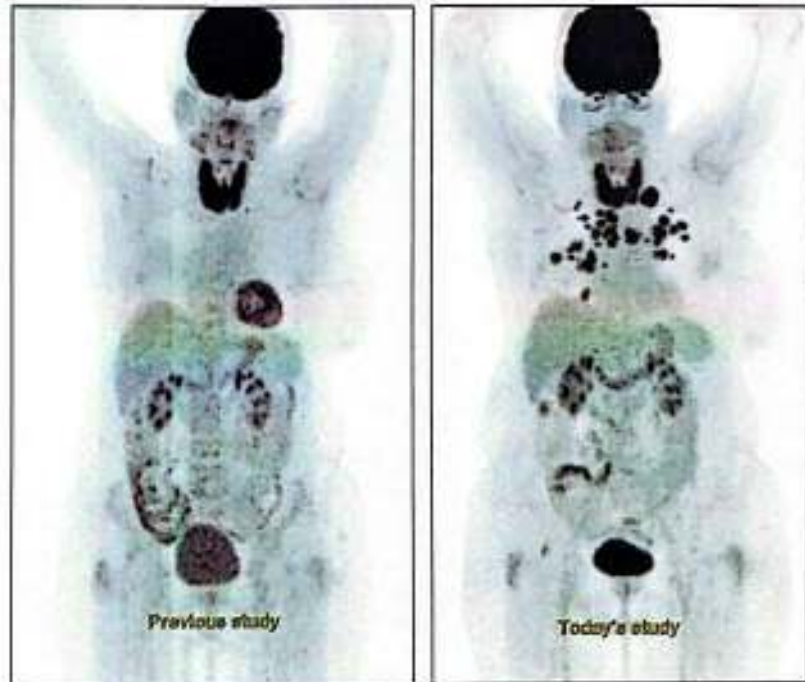
- 6.8 mCi 18F-FDG was administrated to the patient and imaged after appropriate time.
- The blood glucose level was 106 mg/dl.
- A diagnostic 128 MDCT post contrast examination was taken after I.V. non-ionic contrast administration of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

PET/CT FINDINGS:-

Compared to previous PET/CT study dated 14-7-2025, the current study revealed:

A) Head and Neck:-

- The neck is still devoid of any abnormally hypermetabolic lymph nodes.
- The visualized portions of the brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.



أ.د. أحمد
أ.د. محمد
أ.د. عمر
أ.د. وحيد حسين
أ.د. ياسر عبد العظيم عباس
أ.د. أحمد محمد منيب
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

Patient Name: Sohair Ahmed Othman	Patient ID: 1336351
Dear Sir	Study Date: 22/04/2026
Acc No: 2690712	

- Still noted the enlarged thyroid gland with diffuse parenchymal activity achieving currently 11.8 SUVmax (compared to 16.1 SUVmax previously), likely thyroiditis.

B] Chest:-

- The right mastectomy operative bed as well as the left breast parenchyma are devoid of any FDG avid soft tissue density lesions that would account for recurrent neoplasia.
- **Newly developed** hypermetabolic bilateral hilar and mediastinal [pretracheal/retrocaval and aorto-pulmonary window] LNs are noted, measuring up to 18 mm and achieving up to 13.7 SUVmax.
- **Newly developed** multiple variable sized hypermetabolic scattered bilateral pulmonary nodules are noted, the largest and most active is seen at left lung upper lobar apicoposterior segment, based on the mediastinal pleura, measuring up to 26x30 mm and achieving 16.1 SUVmax.
- Inactive subpleural reticulations are seen at right lung upper and middle lobes, likely post radiation sequel.
- Still no pleural or pericardial effusion detected.
- Normal tracer biodistribution along the myocardium is noted.



C] Abdomen & Pelvis:-

- No hypermetabolic abdominal or pelvic lymph nodes detected.
- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.

د. أحمد
د. محمد أبو
د. عمر حسني
د. وحيد حسين ط
د. ياسر عبد العظيم عباس
د. أحمد محمد منيب

أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

Patient Name: Sohair Ahmed Othman	Patient ID: 1336351
Dear Sir	Study Date: 22/04/2026
Acc No: 2690712	

- Physiological uptake in liver, adrenal glands, spleen and pancreas.
- Physiological excretion of FDG in the kidneys.
- No ascites noted.
- Still noted the inactive partially calcified uterine right lateral wall myoma, measuring about 2.2cm.
- Small fat containing umbilical hernia is still noted.

D] Musculoskeletal:-

- Still no hypermetabolic osseous lesions could be detected along the surveyed skeleton.
- Still noted spondylodegenerative changes at multiple levels of the spine.
- Still noted tiny inactive left femoral head lytic osseous lesion, likely benign entity.

CONCLUSION:-

Follow up PET/CT study revealed:

- **Still no evidence of recurrent breast neoplasia.**
- **Newly developed hypermetabolic bilateral pulmonary nodules as well as hilar and mediastinal nodal lesions, mostly metastatic deposits.**

MUCH OBLIGED
PROF. DR. SUSAN ADIL
EA